

Gout Medication UK: A Complete Guide to Drugs for Gout

From managing a gout attack to long-term uric acid control — everything you need to know about gout treatment in the UK.

By **Living With Arthritis Editorial Team** · Pending clinical review ·
2026-05-27 · livingwitharthritis.org.uk

Gout is the most common form of inflammatory arthritis in the UK, affecting an estimated 2.5 million people — and rates have been rising for decades. A gout attack is one of the most painful medical events a person can experience: the intense burning, swelling, and redness, typically in the big toe but potentially in any joint, can be completely debilitating.



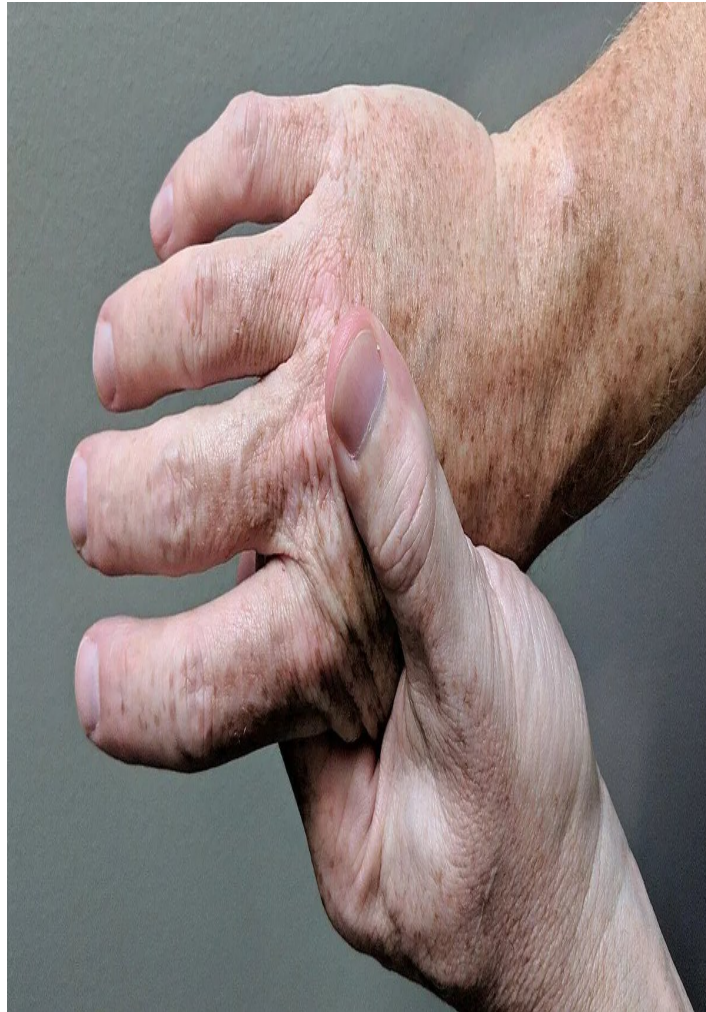
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The good news is that gout is one of the most treatable forms of arthritis. With the right gout medication in the UK, attacks can be stopped quickly, and with consistent long-term treatment, many people become entirely gout-free. This guide covers everything from how gout develops, to the drugs used to treat acute attacks, to the urate-lowering therapies available to UK patients.

Understanding Gout: Why Medication Is Needed

Gout is caused by hyperuricaemia — elevated levels of uric acid in the blood. When uric acid exceeds its solubility threshold (approximately 360 micromol/L), it crystallises as monosodium urate (MSU) crystals in joint spaces and surrounding tissue. These sharp, needle-like crystals trigger an intense immune response, producing the extreme pain and inflammation of a gout attack.

Without treatment, gout attacks become more frequent and eventually chronic. Crystals can also accumulate in soft tissue (tophi), damage kidneys, and cause cardiovascular complications. This is why long-term medication to control uric acid levels — not just treating individual attacks — is essential for most people with recurrent gout.



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Medications for Acute Gout Attacks

The priority during an acute gout attack is to reduce pain and inflammation as quickly as possible. Three types of medication are used.

NSAIDs (Non-Steroidal Anti-Inflammatory Drugs)

NSAIDs are usually the first-line treatment for acute gout in the UK. Naproxen (500 mg twice daily) and indomethacin are the most commonly prescribed. They work by inhibiting COX enzymes and reducing prostaglandin production, thereby rapidly reducing inflammation and pain. Most people notice significant improvement within 24 hours.



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NSAIDs must be used with caution or avoided in people with kidney disease, stomach ulcers, heart failure, or who are taking anticoagulants. Always take NSAIDs with food. Naproxen is available over the counter in the UK at lower doses; higher prescription doses provide greater anti-inflammatory effect.

Colchicine

Colchicine is a plant-derived drug — originally extracted from the autumn crocus — that has been used for gout for over a century. It works by disrupting microtubule function in neutrophils, preventing them from migrating to the inflamed joint and releasing inflammatory mediators.

Modern UK guidelines (BSR and EULAR) recommend low-dose colchicine: 1 mg initially, followed by 500 micrograms one hour later. This low-dose regimen is as effective as the older high-dose approach but with significantly fewer gastrointestinal side effects (nausea, diarrhoea, abdominal cramping). Colchicine is particularly useful for people who cannot

tolerate NSAIDs.

****Important drug interactions for colchicine**** - Statins (increased risk of muscle damage — myopathy). - Ciclosporin (significantly increased colchicine levels — avoid combination). - Clarithromycin and other macrolide antibiotics. - Always inform your GP and pharmacist of all medications you take.

Corticosteroids

When NSAIDs and colchicine are contraindicated or ineffective, oral corticosteroids (typically prednisolone 30–35 mg daily for 3–5 days) are highly effective for acute gout. Intra-articular steroid injections directly into the affected joint provide rapid, targeted relief and are particularly useful for large joints such as the knee or ankle.

Corticosteroids should not be used long-term for gout due to well-known side effects including blood sugar elevation, bone thinning, weight gain, and adrenal suppression.

Long-Term Urate-Lowering Therapy (ULT)

After a confirmed gout diagnosis — particularly with two or more attacks per year, tophi, kidney stones, or chronic kidney disease — UK guidelines recommend starting urate-lowering therapy to permanently reduce uric acid levels. The target is to keep serum urate below 360 micromol/L (and below 300 micromol/L in those with tophi).

Allopurinol — The Gold Standard

Allopurinol is the first-line ULT in the UK and worldwide. It is a xanthine oxidase inhibitor, meaning it blocks the enzyme responsible for the final step in uric acid production. Starting at 100 mg daily and gradually titrating upward (typically to 300 mg, sometimes higher), allopurinol effectively reduces uric acid in the majority of patients.

A key principle: allopurinol must always be started after an acute attack has fully resolved (usually two to four weeks after), never during one, as starting it during an attack can paradoxically prolong or worsen the flare. A prophylactic dose of colchicine (500 mcg once or twice daily) is typically prescribed for the first six months of allopurinol therapy to prevent this.

Allopurinol is generally well tolerated. Rare but serious reactions include allopurinol hypersensitivity syndrome (AHS) — a potentially life-threatening skin reaction. Risk is higher in people of Han Chinese, Thai, and Korean descent, who should be tested for the HLA-B*5801 allele before starting treatment.

Febuxostat — An Alternative for Allopurinol Intolerance

Febuxostat (brand name Adenuric in the UK) is a non-purine xanthine oxidase inhibitor, available in the UK for people who cannot tolerate or do not respond adequately to allopurinol. It is often more potent than standard allopurinol doses and does not require dose adjustment for mild-to-moderate kidney impairment.

A 2019 clinical trial (CARES trial) raised concerns about cardiovascular mortality with febuxostat versus allopurinol, though subsequent European analyses have not replicated this finding. In the UK, febuxostat should be used with caution in people with established cardiovascular disease.

Benzbromarone and Other Uricosurics

Uricosuric drugs work differently — instead of reducing uric acid production, they increase its excretion via the kidneys. Benzbromarone is available on a named-patient basis in the UK for patients who overproduce uric acid or have failed other treatments. Probenecid is another option. These drugs are contraindicated in kidney stone formers as they increase urinary urate and the risk of stone formation.

Quick Answer

For acute gout attacks, UK doctors prescribe NSAIDs (naproxen, indomethacin), low-dose colchicine, or corticosteroids. For long-term prevention, urate-lowering therapy with allopurinol (first-line) or febuxostat is recommended. The target is to keep serum urate below 360 micromol/L. Medication should always be managed by a GP or rheumatologist.

Lifestyle Measures Alongside Medication

Medication is most effective when supported by lifestyle changes: staying well hydrated (aim for 2–3 litres of fluid daily), reducing beer and spirits, limiting red meat and seafood high in purines, avoiding fructose-rich drinks, achieving a healthy weight, and taking any prescribed diuretics (water tablets) only as directed by your doctor, as these can raise uric acid levels.

Conclusion

Gout is eminently treatable and, with the right medication and lifestyle management, entirely preventable. If you are experiencing recurrent attacks, speak to your GP about starting

urate-lowering therapy. Do not wait — untreated gout causes cumulative joint damage that can be difficult to reverse.

Q&A

Question: What is the key thing to know about gout medication uk: a complete guide to drugs for gout?

Short answer: For acute gout attacks, common treatments include NSAIDs like naproxen (500mg twice daily) or indomethacin, which reduce inflammation and pain. Colchicine, taken as 1mg initially then 500 micrograms one hour later, is an alternative, especially if NSAIDs are unsuitable. If these are ineffective, corticosteroids such as prednisolone (30-35mg daily for 3-5 days) can be used.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis · livingwitharthritis.org.uk